RIDGELINE ICU SURGE COMPACT

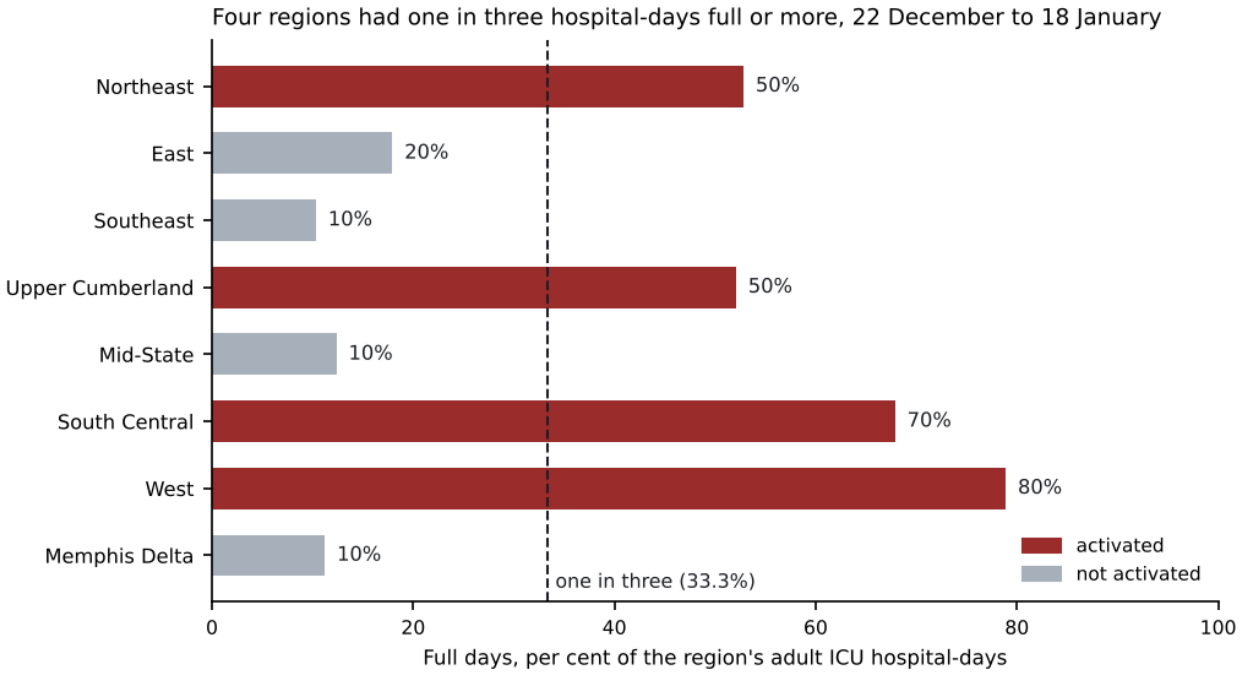
The compact activates in Northeast, Upper Cumberland, South Central and West from 26 January

Activation notice for 26 January to 22 February 2026, from the operations committee's review of Tuesday 20 January. Kaitlin Levy, data team, Ridgeline Hospital Alliance. Issued 21 January 2026.

At its review of 20 January the operations committee activated the compact in Northeast, Upper Cumberland, South Central and West for the four weeks from Monday 26 January to Sunday 22 February 2026: in each of them at least one in three of the region's adult ICU hospital-days from 22 December to 18 January were full days. In East, Southeast, Mid-State and Memphis Delta the share was well under one in three, and the compact is not activated there. Activation brings the transfer, staffing and ambulance arrangements of article 5 into force in the four regions.

Region	Compact hospitals	Hospital-days	Share of hospital-days full	Activated
Northeast	9	252	50%	Yes
East	12	336	20%	No
Southeast	10	280	10%	No
Upper Cumberland	5	140	50%	Yes
Mid-State	14	392	10%	No
South Central	6	168	70%	Yes
West	7	196	80%	Yes
Memphis Delta	12	336	10%	No
All regions	75	2,100		

Shares to the nearest ten per cent; the committee decides on the unrounded share against one in three (article 4.1).



Source: the alliance's census survey, the regional bed-status board and the Ridgeline Transfer Center's call log, extract of 20 January 2026.

How the shares were estimated

None of the three feeds records full days. The census survey counts adult ICU patients against staffed beds at 07:00 and misses a unit that fills later in the day; the bed-status board shows an adult ICU Closed on nearly every full day, but also on many ordinary days, when a hospital keeps its last staffed beds for its own patients; the transfer center shows a no-bed decline only when a request reaches the hospital that day. We conformed the three feeds to the 2,100 hospital-days of the 28 days, on each hospital's own dates: the latest census report of each hospital and day, the adult ICU line's board status replayed from its events, and the adult ICU calls declined for no staffed bed. Each region's share of full days is the prevalence in a two-class latent class model of the three feeds, each feed's chance of showing a full day and an ordinary day the same at every hospital, the share free in each region, fitted by maximum likelihood; a day without a census report (Christmas Day and New Year's Day at some hospitals) enters through the other two feeds. The fit puts the board at about nine in ten full days and a third of ordinary days, the census at about a third of full days and the transfer center at about a quarter, each with few false signals. Counting a day full whenever any feed shows it would put all eight regions over one in three; counting it only when two feeds agree would put none there.

Transfers, diversion and closures by region

For the transfer agreements, the EMS coordinators and the committee's reading of how the strain built, region by region:

Region	Adult ICU placements in another region (22 December to 18 January)	Emergency department diversion, hours (22 December to 18 January)	Adult ICU shown Closed, hours (24 November to 21 December)
Northeast	70	80	752
East	40	133	642
Southeast	32	132	475
Upper Cumberland	45	27	357
Mid-State	49	147	745
South Central	65	46	513
West	58	64	616
Memphis Delta	28	69	623
All regions	387	698	4,723

Placements: adult ICU requests opened on the sending hospital's own dates in the 28 days, accepted by a hospital in another region and not cancelled, by the sending hospital's region (annex A counties; senders outside Tennessee have none); each placement of a patient moved on counts, and a case the new call system took over at its 5 January start counts once. Diversion: an emergency department's Divert status and, from 5 January, when the board shows diversion as Advisory, its Advisory status with the reason EMS diversion. Closed: the adult ICU line's Closed status, board events before 8 December read through the facility codes then in use. Hours on each hospital's own clock, added over the region's hospitals and rounded to the whole hour. Source: the Ridgeline Transfer Center's two call logs and the regional bed-status board, extract of 20 January 2026.

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